Article Title	**Efficacy of a community- versus primary care-centered program for childhood obesity: TX CORD RCT**	
Authors	Butte, N., Hoelscher, D., Barlow, S., Pont, S., Durand, C., Vandewater, E., Liu, Y., Adolph, A., Pérez, A., Wilson, T., Gonzalez, A., Puyau, M., Sharma, S., Byrd-Willams, C., Oluyomi, A., Huang, T., Finkelstein, E., Sacher, P. & Kelder, S.	
		Page Number (Location in Text)
Brief Name		
Provide the name or a phrase that describes the intervention.	TX CORD: Texas Childhood Obesity Demonstration project **Intervention:** MEND2-5 (Mind, Exercise, Nutrition…Do it!) program **Comparison**: Next Steps program	
Why		
Describe any rationale, theory or goal of the elements essential to the intervention	The TX CORD project was designed to address childhood obesity through an integrated, systems-oriented model that incorporated primary and secondary obesity interventions. Within this population-level, systems-based intervention was a 12-month randomized controlled trial targeting low-income, ethnically diverse children with overweight or obesity and their families. It is hypothesized that the 12-month, community-centered program would significantly reduce BMI compared to the health care-centered program.	1585 (1)
What		
Materials: Describe any physical or informational materials used in the intervention, including those provided to participants or used in intervention delivery or in training of intervention providers. Provide information on where the materials can be accessed (e.g. online appendix, URL).	**Intervention :** Conducted at the YMCA within 5 miles of the participants. Education materials surrounding general nutrition, PA and behaviours were needed (not included in paper/protocol). Based on CATCH program. All programs were delivered in English or Spanish. A digital scale and stadiometer for weight and height measurements. Transition program includes the book *A Year of Being Well* (role models stories of low-income families using CDC obesity prevention behaviours), cooking classes	74-76 (2)

	based on *The Happy Kitchen/La Coicina Alegre* and MED World online/print material. **Control:** Electronic medical record at the primary care physician's office. Counselling materials for the primary care provider. Activity booklet for parents and children. Material was available in both English and Spanish, and available through the AAP website (https://shop.aap.org/next-steps-a-practitioners-guide-for-themed-follow-up-visits-for-their-patients-to-achieve-a-heal/).	
Procedures: Describe each of the procedures, activities, and/or processes used in the intervention, including any enabling or support activities.	**Intervention:** A multicomponent, healthy lifestyle program including behavioural, nutrition and physical activity sessions. Included 10 weekly 90-minute sessions relating to general nutrition, physical activity and behaviours for children including: - 30 minutes of guided active play - 15 minutes of healthy snack time evidence-based, exposure-based technique to promote acceptance and increased intake of fruit and vegetables - 45 minutes where children participate in creative play activities while parents attend an interactive education and skill development session based on group-based parent training principles. The transition program includes monthly 90-minutes post-program sessions that children and parents attend together. Sessions include a brief review of the MEND concepts, role model stories and cooking lessons. **Control:** A clinic-based program that optimizes health care procedures to assess BMI and provides brief counselling materials to support family/child self-improvements in healthy lifestyles. Electronic health record (BMI screening, alerts for BMI ≥ 85th centile, specific diagnostic codes, lab tests and referrals common in obesity management) changes to support childhood obesity visits. Counselling materials were provided to the primary care providers. Next Steps activity booklets for parents and children to work on nutrition and physical activity targets in a self-directed manner. Families were encouraged to seek follow up clinic visits to address	74-75 (2) 75 (2), 1586 (1)

	childhood obesity if identified as being obese. The brief counselling materials include information on facilitating clinical intervention and supporting educational materials for families.	
Who Provided		
For each category of intervention provider (e.g. psychologist, nursing assistant), describe their expertise, background and any specific training given.	**Intervention**: All sessions were delivered by bilingual allied health professionals (dieticians and health educations) and were assisted by community health workers. Community health workers were trained in MEND, motivational interviewing, nutrition and physical activity programming. **Control:** Program was delivered by primary care physicians in their offices, and self-directed learning by the families/children. No specific training, however counselling material was provided to the primary care physicians Trained research staff measured weight and height (?unclear training)	75 (2) 1586 (1)
How		
Describe the modes of delivery (e.g. face-to-face or by some other mechanism, such as internet or telephone) of the intervention and whether it was provided individually or in a group.	**Intervention:** The intervention was delivered at the YMCA face-to-face. ? It was not stated whether this was individual sessions or group sessions. Measurements were performed at the Children's Nutrition Research Centre in Houston or YMCAs/offices in Austin. **Control:** Delivered face-to-face in primary care physician's office, as well as workbooks provided to complete individually at home.	1585 (1) 1585 (1)
Where		
Describe the type(s) of location(s) where the intervention occurred, including any necessary infrastructure or relevant features.	**Intervention:** conducted at YMCAs in Austin and Houston. Measurements were performed at the Children's Nutritional Research Centre in Houston or YMCAs/offices in Austin. ?No information regarding how participants accessed this **Control**: primary care clinics in Texas	1585 (1)

When and How Much		
Describe the number of times the intervention was delivered and over what period of time including the number of sessions, their schedule, and their duration, intensity or dose.	**Intervention:** The intensive phase consisted of nine weekly 90-minute sessions. The transition phase offered 90-minutes session including MEND reviews (nine months in total). In total, MEND2-5 offered 27 hours of contact. **Intervention:** A clinic-based program – families were provided with self-directed education material as well as encouraged to seek follow-up clinic visits to address childhood obesity. An estimated 8-hours of contact was provided.	1585 (1), 75 (2) 1585-1586 (1)
Tailoring		
If the intervention was planned to be personalised, titrated or adapted, then describe what, why, when, and how.	"extensively adapted for the US population" (from British study) - ?did not report how it was adapted "pilot tested program elements and used qualitative data to further tailor the program to our population" TX CORD culturally adapted Next Steps material for low-income, urban, black and Latino children in both English and Spanish versions.	1585 (1) 76 (2)
Modifications		
If the intervention was modified during the course of the study, describe the changes (what, why, when, and how).	N/A	
How Well		
Planned: If intervention adherence or fidelity was assessed, describe how and by whom, and if any strategies were used to maintain or improve fidelity, describe them.	Fidelity to the session content and participant engagement was assessed using instructor checklists. Compliance in the intervention arm was based on attendance (the number of sessions attended out of the number offered). Retention was based on completion of measurements at 3 and 12 months.	1587 (1)
Actual: If intervention adherence or fidelity was assessed, describe the extent to which the intervention was delivered as planned.	Retention rates were 64% for the intervention group compared to 87% for the comparison at 3 months. The retention rate at 12 months was 54% for the intervention group compared to 82% for the comparison group.	1587-1588 (1)

	During the Intensive Phase, 86% of the sessions were delivered completely as planned. During the Transition Phase, 75% of the sessions were delivered completely as planned. Mean attendance was 46% during the Intensive Phase and 16% during the Transition Phase. Fidelity was not assessed for the comparison group.	1590 (1)

** **Authors -** use N/A if an item is not applicable for the intervention being described. **Reviewers** – use '?' if information about the element is not reported/not sufficiently reported.

(1) Butte, NF, Hoelscher, DM, Barlow, S, et al. Efficacy of a community- versus primary care-centered program for childhood obesity: TX CORD RCT. Pediatric Obesity 2017; 25:1584-1593

(2) Hoelscher DM, Butte NF, Barlow S, et al. Incorporating primary and secondary prevention approaches to address childhood obesity prevention and treatment in a low-income, ethnically diverse population: study design and demographic data from the Texas Childhood Obesity Research Demonstration (TX CORD) study. Child Obes 2015;11:71-91.

Article Title	**A parent treatment program for preschoolers with obesity: A randomized controlled trial**	
Authors	Ek, A., Chamberlain, K., Sorjonen, K., Hammar, U., Malek, Mahnoush., Sandvik, P., Somaraki, M., Nyman, J., Lindberg, L., Nordin, K., Ejderhamn, J., Fisher, P., Chamberlain, P., Marcus, C., & Nowicka, P.	
		Page Number (Location in Text)
Brief Name		
Provide the name or a phrase that describes the intervention.	More or Less Study: 1) Parent-only group treatment **with** follow-up booster sessions 2) Parent only group treatment **without** booster sessions 3) Standard treatment	2 (1)
Why		
Describe any rationale, theory or goal of the elements essential to the intervention	Given the significant role of parents in a young child's life, the treatment program created was grounded in evidence-based parenting practices to evaluate whether a focus on parenting would be especially suitable for this age group as suggested by previous papers. It is hypothesized that the parent-only treatment would be more effective compared to standard treatment.	2 (1)
What		
Materials: Describe any physical or informational materials used in the intervention, including those provided to participants or used in intervention delivery or in training of intervention providers. Provide information on where the materials can be accessed (e.g. online appendix, URL).	**Intervention** - Based on the KEEP (Keeping Foster and Kin Parents Supported and Trained) program – a positive parenting program. The More or Less program was developed in collaboration with the KEEP program developers with conceptual and cultural adaptations designed for Swedish population. Involved home assignments (not provided as part of paper/protocol) In collaboration with the KEEP developers, the authors translated and adapted the parent group manual outlining the content of each session and parent handouts (not included in the protocol/paper).	2 (1) 6 (2)

	Control -receive treatment as usual in an outpatient pediatric unit. A questionnaire was developed to monitor the healthcare providers involved (as it varied), and the number and focus of the visits. The questionnaire was not included as part of the protocol or paper.	6 (2)
Procedures: Describe each of the procedures, activities, and/or processes used in the intervention, including any enabling or support activities.	**Intervention -** ML program consisted of 10 weekly 90-minute sessions with parents only, with childcare provided. Each session centered around a parenting practice as well as a lifestyle component aimed to provide parents with knowledge/skills to support a healthy home environment. The session was finished with a home practice assignment related to the focus of the day. If parents missed a session, material was sent home and key parts were reviewed over the phone by 1 of the group leaders. **More or Less with Booster** – Group leaders called participants every 4-6 weeks for a 30-minute phone call (max 7 calls). The parents were referred back to the discussions had during the program and to the manual for additional support **Control** – based on the action plan for childhood obesity in Stockholm County (not described in the paper). Received at least 4 treatment visits, typically the first with the physician, and subsequent with the pediatric nurse. The visits centered around lifestyle advice. Reasons for treatment are discussed including weight gain and future risk factors. Treatments varied between pediatric units – a questionnaire was included regarding questions about the professions of the involved healthcare providers, number and focus of visits.	3 (1) 6-8 (2) 2(1) 6 (2)
Who Provided		
For each category of intervention provider (e.g. psychologist, nursing assistant), describe their expertise, background and any specific training given.	**Intervention** – Led by 5 registered dieticians – they received training by the KEEP developers, who also provided weekly supervision on videotaped sessions (translated from Swedish to English) for program certification and fidelity. The initial training was a 5-day experiential training including information on program's theory and practice in the delivery of group sessions. Each session of the ML program was thoroughly discussed, and each trainee role played facilitating several key sessions.	3 (1) 7 (2)

	The training continued through external supervision from the KEEP developer group. **Control** – first and last visit were with a pediatrician, and the remaining visits were with a pediatric nurse (minimum of 4 visits). Some children were referred to dieticians and/or PTs.	3 (1)
How		
Describe the modes of delivery (e.g. face-to-face or by some other mechanism, such as internet or telephone) of the intervention and whether it was provided individually or in a group.	**More or Less without Boosters**– face-to-face group sessions **More or Less with booster** – face-to-face group sessions followed by seven 30-minute phone calls every 4-6 weeks (9 months in total) Neither the group leaders or parents were informed about which families were allocated to the boosters until after the group sessions were finished in order to avoid differential treatment. **Control** – delivered in person during routine visits to pediatrician	3 (1)
Where		
Describe the type(s) of location(s) where the intervention occurred, including any necessary infrastructure or relevant features.	**Intervention -** Meetings were held in health care facilities. Both parents were invited to participate and childcare was provided. **Control** – Delivered at 14 outpatient pediatric clinics	3 (1)
When and How Much		
Describe the number of times the intervention was delivered and over what period of time including the number of sessions, their schedule, and their duration, intensity or dose.	**No Booster** – 10 weekly 90-minute sessions **Booster** – 10 weekly 90-minute sessions with 30-minute phone calls (max 7) every 4-6 weeks for the following 9 months. **Control** – Were offered ≥ 4 individual visits of around 30 minutes over 12 months. Treatment setup varied between clinics – each clinic completed a protocol at every visit documenting details (*not available in paper*)	3 (1) 6 (2)
Tailoring		

If the intervention was planned to be personalised, titrated or adapted, then describe what, why, when, and how.	**Intervention** – The parent group manual outlining each session and parent handouts was translated and adapted from the original KEEP developers' version. **Control** – treatment set up varied between practitioners and location. Some children were referred to dieticians and/or PTs. Number of appointments with pediatrician/pediatric nurse also varied.	6 (2)
Modifications		
If the intervention was modified during the course of the study, describe the changes (what, why, when, and how).	A 6-month follow-up was added post baseline to better evaluate the treatment program	2 (1)
How Well		
Planned: If intervention adherence or fidelity was assessed, describe how and by whom, and if any strategies were used to maintain or improve fidelity, describe them.	**Intervention** - Training of the ML group leaders was conducted by the KEEP developers. 5-day experiential training session. The training continued through external supervision from the KEEP group – all parent groups were videotaped (and translated to English). KEEP consultants viewed the recordings, rated them for fidelity and identified areas for reinforcement and feedback Group leaders completed a session review with questions regarding what went well and challenges experienced. Also completed weekly forms on parent attendance and engagement ratings – these measures informed the consultation process **Control -** Because the treatments vary between pediatric units, a questionnaire including questions about the profession of healthcare providers involved as well as the number and focus of visits	7 (2) 6 (1)
Actual: If intervention adherence or fidelity was assessed, describe the extent to which the intervention was delivered as planned.	**Intervention**– Not reported in the final paper. **Control** – Each clinic filled out a protocol at every visit documenting what treatment the family had attended and on how many occasions.	3 (1)

** **Authors** - use N/A if an item is not applicable for the intervention being described. **Reviewers** – use '?' if information about the element is not reported/not sufficiently reported.

(1) Ek A, Lewis Chamberlain K, Sorjonen K, et al. A Parent Treatment Program for Preschoolers With Obesity: A Randomized Controlled Trial. Pediatrics. 2019;144(2): e20183457

(2) Ek A., Lewis Chamberlain K., Ejderhamn J., et al. The more or less study: A randomized controlled trial testing different approaches to treat obesity in preschoolers. Public Health. 2015. 15:735: DOI 10.1186/s12889-015-1912-1

Article Title	Two-year follow-up of a primary care-based intervention to prevent and manage childhood obesity: the High Five for Kids study (1)	
Authors	Sheryl L. Rifas-Shiman, Elsie M. Taveras, Steven L. Gortmaker, Katherine H. Hohman, Christine M. Horan, Ken P. Kleinman, Kathleen Mitchell, Sarah Price, Lisa A. Prosser and Matthew W. Gillman	
		Page Number (Location in Text)
Brief Name		
Provide the name or a phrase that describes the intervention.	High Five for Kids primary care-based pre-schooler obesity intervention	
Why		
Describe any rationale, theory or goal of the elements essential to the intervention	Targeting preschool-age children may be beneficial because children of this age visit their primary care clinicians frequently for routine medical care and parents have more control over their children's health-related behaviours, and these behaviours may be more malleable. The purpose of the study was to assess the effectiveness of the primary care-based obesity intervention over a 2-year period. The overarching model for this intervention was the Chronic Care Model which posits that changes in primary care to produce functional patient outcomes requires changes for all members of the practice team.	24 (2) 2 (1)
What		
Materials: Describe any physical or informational materials used in the intervention, including those provided to participants or used in intervention delivery or in training of intervention providers. Provide information on where	**Intervention:** Posters were developed for the waiting rooms that encouraged the five target behaviours of the High Five. An image of this poster is available at: https://pubmed.ncbi.nlm.nih.gov/21464376/. Educational modules were developed to target fast-food and sugar-sweetened beverage consumption, as well as television viewing (https://www.researchgate.net/publication/261773967_Health_Behavior_Change_A_Guide_For_Practitioners). A list of local resources for physical activity was also available but not provided in the study write-up, as was a website with recipes and educational materials. Small physical incentives (water bottles, snack containers, books) were provided by nurse practitioners to participants, as was an	2-3 (2)

the materials can be accessed (e.g. online appendix, URL).	electronic tool to monitor television-viewing time (more information on this tool, including where to get it, was not provided in the study). The electronic medical record was enhanced to help physicians make decisions, track patients, follow-up, schedule and bill.	
Procedures: Describe each of the procedures, activities, and/or processes used in the intervention, including any enabling or support activities.	**Intervention:** Intervention was based on the Chronic Care Model (3), which requires change from all members of the health care team. Participants (including families) received motivational interviewing by nurse practitioners and educational modules targeting certain behavioural modifications. Specifically, the interviewing and modules targeted reducing sugar-sweetened beverage intake, reducing television viewing time, increasing fruit and vegetable consumption, decreasing fast-food consumption and encouraging more time in active play. **Usual care:** Participants got the standard of care offered by their pediatric practice. This included the baseline and annual well-child care visits as well as follow ups with the paediatrician or subspecialist (ex. Dietician).	2 (2)
Who Provided		
For each category of intervention provider (e.g. psychologist, nursing assistant), describe their expertise, background and any specific training given.	**Intervention:** Pediatric nurse practitioners were trained for their role as the central intervening clinician. They also received training in motivational interviewing. This training included enhancing motivation for change, enhancing self-efficacy, how to help recognize inconsistencies between desired and actual behaviours, and how to help reduce this dissonance. Medical assistants were trained on how to conduct research-standard anthropometric measurements (for collecting height and weight data). Medical receptionists were trained to schedule the initial and follow-up visits with the nurse practitioners in accordance with the study protocol. Primary care pediatricians were trained in obesity management and to use brief, focused negotiation skills. **Usual care:** Participants got the standard of care offered by their pediatric practice. This included the baseline and annual well-child care visits as well as follow-ups with the pediatrician or subspecialist (ex. Dietician).	2 (2)
How		
Describe the modes of delivery (e.g. face-to-face or by some other mechanism, such as internet or telephone)	**Intervention:** In the first year, four visits were face-to-face and two were by telephone. In the second year, both visits were face-to-face. Visits were with individual families. **Usual care:** At the discretion of the participants regular pediatrician.	2 (2)

of the intervention and whether it was provided individually or in a group.		
Where		
Describe the type(s) of location(s) where the intervention occurred, including any necessary infrastructure or relevant features.	Done at 10 pediatric primary care practices within the Harvard Vanguard Medical Associates, which is a multi-site group practice in Massachusetts. Half of the practices provided usual care and half provided the intervention care.	2 (2)
When and How Much		
Describe the number of times the intervention was delivered and over what period of time including the number of sessions, their schedule, and their duration, intensity or dose.	**Intervention:** Four in person (each 25 minutes) visits with the clinician over the first year, as well as two phone calls (15 minutes each). Two in-person visits in the maintenance year (length not described). **Usual care:** Frequency of usual care visits was determined by the routine care procedures for each particular pediatric practice.	2 (2)
Tailoring		
If the intervention was planned to be personalised, titrated or adapted, then describe what, why, when, and how.	**Intervention:** During their visits, the families could choose to work on one or more of the behavioural targets (previously listed).	4 (1)
Modifications		
If the intervention was modified during the course of the study, describe the changes (what, why, when, and how).	N/A	
How Well		

Planned: If intervention adherence or fidelity was assessed, describe how and by whom, and if any strategies were used to maintain or improve fidelity, describe them.	**Intervention:** Adherence was assessed by completion of the various visits/phone calls, during data analysis. No strategies were used to improve fidelity.	5 (2), 3 (1)
Actual: If intervention adherence or fidelity was assessed, describe the extent to which the intervention was delivered as planned.	**Intervention:** In the first year (the intervention period), only 56% of participants in the intervention completed at least 2 of the 6 intervention activities (in person or phone visits). During the second year (the maintenance period), only 17% went to one visit and only 21% of participants went to both visits. Overall adherence was thus low. **Usual care:** adherence was not measured for usual care participants.	5 (2), 3 (1)

** **Authors** - use N/A if an item is not applicable for the intervention being described. **Reviewers** – use '?' if information about the element is not reported/not sufficiently reported.

References:

1. Rifas‐Shiman, S. L., Taveras, E. M., Gortmaker, S. L., Hohman, K. H., Horan, C. M., Kleinman, K. P., ... & Gillman, M. W. (2017). Two‐year follow‐up of a primary care‐based intervention to prevent and manage childhood obesity: the High Five for Kids study. *Pediatric obesity*, *12*(3), e24-e27.

2. Taveras, E. M., Gortmaker, S. L., Hohman, K. H., Horan, C. M., Kleinman, K. P., Mitchell, K., ... & Gillman, M. W. (2011). Randomized controlled trial to improve primary care to prevent and manage childhood obesity: the High Five for Kids study. *Archives of pediatrics & adolescent medicine*, *165*(8), 714-722.

3. Wagner, E. H. (1998). Chronic disease management: what will it take to improve care for chronic illness?. *Effective clinical practice*, *1*(1).

Article Title	Clinic and Home-Based Behavioral Intervention for Obesity in Preschoolers: A	

	Randomized Trial (1)	
Authors	Lori J. Stark, PhD, Stephanie Spear Filigno, PhD, Christopher Bolling, MD, Megan B. Ratcliff, PhD, MPH, Jessica C. Kichler, PhD, Shannon M. Robson, PhD, MPH, RD, Stacey L. Simon, PhD, Mary Beth McCullough, PhD, Lisa M. Clifford, PhD, Cathleen Odar Stough, PhD, Cynthia Zion, BS, and Richard F. Ittenbach, PhD	
		Page Number (Location in Text)
Brief Name		
Provide the name or a phrase that describes the intervention.	Comparing LAUNCH (Learning about Activity and Understanding Nutrition for Child Health), motivational interviewing and standard care as in intervention for pre-school obesity	
Why		
Describe any rationale, theory or goal of the elements essential to the intervention	Obesity in preschool years increases the risk of being overweight and obese in later childhood through to adulthood. Focusing on children with obesity is important as they are at higher risk for mortality compared with overweight peers. This study hypothesizes that an skills-based behavioural family clinic in combination with a home-based intervention would reduce BMIz score compared with motivational interviewing and standard care in preschool age children with obesity.	1,2 (1)
What		
Materials: Describe any physical or informational materials used in the intervention, including those provided to participants or used in intervention delivery or in training of intervention providers. Provide information on where the materials can be accessed (e.g. online appendix, URL).	**LAUNCH**: Caregivers were given an iPod Touch so that they could use the "DietTracker" application to track the child's food and beverage intake. Pedometers were also provided to the children and their caregivers, to help caregivers monitor physical activity levels. Caregivers were also given a two-week supply of fresh vegetables at each of the clinic sessions to help expose children to new vegetables. **Motivational Interviewing:** Caregivers were given a pack of informational handouts from the Let's Go 5-2- 1-0 program (https://www.mainehealth.org/Lets-Go/Childrens-Program/Parents). A motivational interviewing (MI) manual was used	13, 16 (2), 135 (4)

	for MI review by interviewers and study staff (3). **Standard Care:** N/A	
Procedures: Describe each of the procedures, activities, and/or processes used in the intervention, including any enabling or support activities.	**LAUNCH**: Sessions alternated between clinic and home-based sessions. During the clinic sessions, the caregiver(s) and child were in different groups. Caregiver groups received education and problem-solving skills on a number of diet and physical activity, as well as strategies to manage the behaviour of their child. A list of topics by session is available in Table 1 (2). In following the teachings of the sessions, families aimed to decrease the child's screen time, increase physical activity (for which they had daily step goals for both the parents and the child), stay within the prescribed daily calorie goals for the child as well as choose healthier foods, and to record child's food and beverage intake. The behavioural strategies that they learned to help achieve these goals included praising their child's healthy dietary and physical activity choices, modelling healthy behaviours, managing tantrums by ignoring and using time-outs, contingency management, as well as how to best set up the food environment to encourage healthy choices (ex. Not having unhealthy food available). More information about the teaching sessions and strategies learned is available here (4). The child group included education about healthy eating and opportunities for physical exercise. Sessions also included a light nutritious meal provided to caregivers and children, which served as exposure to fruits and vegetables for the children. During home visits, families got a chance to practice and receive feedback on the skills they had learned in clinic, as well as to problem solve any barriers they were facing in implementing those strategies in the home setting. **Motivational Interviewing:** MI was a caregiver-only intervention. During the first session, caregivers were asked about their concerns for their child's diet, weight, or physical activity habits and then given a chance to set a goal that was meaningful to	4-5 (1), 13-15 (2), 135 (4)

	them. The subsequent sessions (either by phone or in person) involved a discussion of the previous goals, how their implementation was going, enhancing motivation, working on establishing the desired behaviours and choosing new goals to work towards. **Standard Care:** No intervention was provided outside of the routine pediatric care the participant was already receiving, other than to be notified of their weight.	
Who Provided		
For each category of intervention provider (e.g. psychologist, nursing assistant), describe their expertise, background and any specific training given.	**LAUNCH**: The LAUNCH interventionists were therapists trained on the protocol in pilot studies (more information on training was not provided). A new LAUNCH interventionist was brought on midway through the study. She had extensive previous training in pediatric behavioural weight management and was trained on the LAUNCH protocol by shadowing and co-leading sessions with one of the previous interventionists. Home visits were completed by postdoctoral fellows, who were trained by reviewing the LAUNCH manual (not provided), listening to audiotapes of home sessions, watching videotapes of child group sessions, administering role-played sessions, and shadowing a second year fellow during home visits for one cycle before being assigned their own visits. The group-sessions in the clinic were done by a licensed clinical psychologist. Fidelity coders had PhDs and expertise in obesity treatment. **Motivational Interviewing:** Interviewers were either pediatricians or PhD level psychologists. The first session was done by the pediatrician and the following sessions were done by the psychologists. Interviewers were selected for their previous MI training and experience; each interviewer had approximately 10 years of MI experience and had participated in previous structured MI training. **Standard Care:** Standard care was provided by the participant's regular pediatrician.	13, 16 (2)
How		
Describe the modes of delivery (e.g. face-to-face or by some other mechanism, such as internet or	**LAUNCH**: All sessions were done in-person. The clinic sessions were done in groups, with one group for the parents and one group for the children. Home-based sessions	13 (2)

telephone) of the intervention and whether it was provided individually or in a group.	were done with individual families. **Motivational Interviewing:** There were 4 in-person sessions (sessions 2, 12, 16) and 14 sessions done over the phone. All sessions were done with individual caregivers. **Standard Care:** Participants in the standard care group received only a notification of their child's weight, provided by phone call and during the initial (in-person) assessment.	
Where		
Describe the type(s) of location(s) where the intervention occurred, including any necessary infrastructure or relevant features.	**LAUNCH**: The home sessions were done in the participants' homes, which were all within 50 miles of the treatment centre. The clinic sessions were done at a medical centre (the treatment centre). **Motivational Interviewing:** The initial assessment interview was done at the medical centre, but subsequent sessions were done at the families' homes or over the telephone. **Standard Care:** At the discretion of the participants' regular pediatricians.	117 (1)
When and How Much		
Describe the number of times the intervention was delivered and over what period of time including the number of sessions, their schedule, and their duration, intensity or dose.	**LAUNCH**: Intervention sessions alternated between clinic (10 sessions) and home sessions (8 sessions), for a total of 18 sessions. For the first three months there were weekly sessions, and for the following three months sessions were every second week. The clinic-based sessions were each 90 minutes and the home visits were 60 minutes. Sessions were scheduled for the late afternoons/early evenings to minimize scheduling conflicts/barriers. **Motivational Interviewing:** The 18 sessions were delivered in alignment with the LAUNCH session schedule. Phone sessions varied in length depending on caregiver preference, but were generally between 15-35 minutes long. Home visits were all scheduled for 60 minutes.	13, 15 (2)

	Standard Care: Length and frequency of appointments was determined by the participant's normal pediatrician, in line with their practice policies.	
Tailoring		
If the intervention was planned to be personalised, titrated or adapted, then describe what, why, when, and how.	**LAUNCH**: Home visits provided the opportunity to tailor the program for individual children/families. If there was a barrier to using a clinic-taught strategy in the home, the home visit provided an opportunity to problem-solve. It was also an opportunity for targeted feedback, since therapist was able to witness the taught behavioural strategies being used in vivo. Caregivers also received weekly written feedback on their child's food and beverage consumption, including feedback on their progress towards their goals. Feedback was also provided on activity levels, based on the number of steps recorded on the pedometers. **Motivational Interviewing:** Caregivers were asked to select a specific nutrition or physical activity goal from the Let's Go 5-2-1-0 (5 fruits or vegetables a day, limit recreational screen time to 2 hours or less, get 1 or more hours of physical activity a day, limit or eliminate sugary drinks) and AAP recommendations (https://pediatrics.aappublications.org/content/120/Supplement_4/S164). This was then used as the focus of the motivational interviewing sessions. **Standard Care:** Care was personalized as appropriate by the pediatrician providing care.	13 (2)
Modifications		
If the intervention was modified during the course of the study, describe the changes (what, why, when, and how).	**LAUNCH**: N/A **Motivational Interviewing:** N/A **Standard Care:** N/A	
How Well		
Planned: If intervention adherence or fidelity was assessed, describe how and by whom, and if any strategies were used to maintain or improve fidelity, describe them.	All LAUNCH and MI sessions were videotaped and/or audiotaped. **LAUNCH**: Fidelity coders reviewed the first session of the first cycle of LAUNCH and provided feedback to the interventionist. One quarter of all LAUNCH sessions (both clinic and home-based) were compared to a LAUNCH checklist by a PhD level	16 (2)

	consultant with experience in weight modification treatment. If fidelity was found to drop below 90%, the therapist was provided with feedback on the components that needed improvement. **Motivational Interviewing:** For the first three cycles of interviewing, the two interviewers met together to review audiotapes of each other's MI interviews and provide feedback on MI fidelity. After the third interviewer joined the study, her audiotapes were reviewed and given regular feedback by the other two interviewers. MI fidelity coders also reviewed audiotapes regularly throughout the study and provided feedback to each interviewer on a quarterly basis. 25% of MI sessions were compared to a MI checklist (3) to assess fidelity. If fidelity was found to drop below 90%, the therapist was provided with feedback on the components that needed improvement. **Standard Care:** N/A	
Actual: If intervention adherence or fidelity was assessed, describe the extent to which the intervention was delivered as planned.	**LAUNCH**: Fidelity was assessed as planned – see previous box. **Motivational Interviewing:** Fidelity was assessed as planned – see previous box. **Standard Care:** N/A	16 (2)

** **Authors** - use N/A if an item is not applicable for the intervention being described. **Reviewers** – use '?' if information about the element is not reported/not sufficiently reported.

References:

1. Stark, L. J., Filigno, S. S., Bolling, C., Ratcliff, M. B., Kichler, J. C., Robson, S. M., ... & Zion, C. (2018). Clinic and home-based behavioral intervention for obesity in preschoolers: a randomized trial. *The Journal of pediatrics*, *192*, 115-121.

2. Stark, L. J., Filigno, S. S., Bolling, C., Ratcliff, M. B., Kichler, J. C., Robson, S. L., ... & Zion, C. (2017). Learning about Activity and Understanding Nutrition for Child Health (LAUNCH): Rationale, design, and implementation of a randomized clinical trial of a family-based pediatric weight management program for preschoolers. *Contemporary clinical trials*, *52*, 10-19.

3. Moyers, T. B., Martin, T., Manuel, J. K., Miller, W. R., & Ernst, D. (2010). Revised global scales: Motivational interviewing treatment integrity 3.1. 1 (MITI 3.1. 1). *Unpublished manuscript, University of New Mexico, Albuquerque, NM*.

4. Stark, L. J., Spear, S., Boles, R., Kuhl, E., Ratcliff, M., Scharf, C., ... & Rausch, J. (2011). A pilot randomized controlled trial of a clinic and home‐based behavioral intervention to decrease obesity in preschoolers. *Obesity*, *19*(1), 134-141.

Article Title	A child-centered health dialogue for the prevention of obesity in child health services in Sweden – A randomized controlled trial including an economic evaluation	
Authors	Derwig, M., Tiberg, I., Björk, J., Welander Tärneberg, A., & Kristensson	

	Hallström, I.	
		Page Number (Location in Text)
Brief Name		
Provide the name or a phrase that describes the intervention.	Child-Centered Health Dialogue (CCHD)	Pg. 2 (1)
Why		
Describe any rationale, theory or goal of the elements essential to the intervention	The attention paid to dietary and physical activity behaviours in children is infrequent at the 4-years health visit in Sweden and raises concern about the efficacy of prevention efforts against child obesity. Nurses working in child health services lack guidance for the management of pre-school children with overweight, which results in a number of consequences that vary from unnecessary extra visits (continuous observation) to not identifying overweight at all. Need for continuous training in the use of structure person-centered family counselling empowering families when a child is identified with overweight. A structured model was developed for the prevention of obesity that can be implemented in routine practices in CHS.	Pg. 2 (1)
What		
Materials: Describe any physical or informational materials used in the intervention, including those provided to participants or used in intervention delivery or in training of intervention providers. Provide information on where the materials can be accessed (e.g. online appendix, URL).	**Control:** Guided using the digital National Handbook for Child Health (*get reference)*. Illustrations were developed for the CCHD were published in the national handbook. A questionnaire to describe usual care was distributed at the start of the RCT. **Intervention:** Illustrations (*not provided*). BMI growth charts were also used in discussion with children and their parents.	Pg. 3 (1)
Procedures: Describe each of the procedures, activities, and/or processes used in the intervention, including any enabling or support activities.	**Control:** Received the 4-years health visit including a health dialogue and identification of overweight using the IOTF definition. The nurses working in child health services are guided using the digital National Handbook of Child Health Services. Illustrations were published in the national handbook to help guide education – overview of the illustrations available (2). A questionnaire to describe the usual care was distributed before the start of the RCT. As child health services is	Pg. 3 (1), 387 (2)

	not responsible for treatment of diseases, children with obesity are referred to a multidisciplinary team consisting of a specialized nurse, dietician and a pediatrician for treatment outside the primary care setting. **Intervention:** 4-year-old children received CCHD at the 4-years health visit. Children with overweight were offered the targeted part of CCHD and extra visits or a referral to an overweight team, dietician or CHS psychologist at the caregiver's request. Children with obesity were referred to specialized care (described under usual care). CCHD is a low-intense multicomponent child-centered intervention based on the Logic model (2). Builds on a family-therapeutic solution-focused approach while emphasizing a relationship based on trust and facilitation of the family to make healthy choices. Consists of 2 parts: Universal part directed at all 4-year-olds regardless of weight and their families, and a targeted part offered to families when the child is identified with overweight. *Universal:* 10-minute structured dialogue between the child, caregiver and nurse using 8 animated illustrations (2) based on healthy choices (fruit/veg consumption drinking water, portion sizes, physical activity, tooth brushing and bedtime routines). As well as a neutral discussion re child's growth using BMI growth charts. *Targeted:* 45-minute sessions, Family Guidance is offered 1-3 weeks after the universal health visit, offered to caregivers, the child and other adults important to the family. Inspired by the Standardized Obesity Family Therapy – an evidence-based treatment model that engages family members through collaborative family support in either identifying protective factors or finding sustainable changes in their lifestyle.	Pg. 3(1), 386 (2)
Who Provided		
For each category of intervention provider (e.g. psychologist, nursing assistant), describe their expertise, background and any specific training given.	**Control:** Guided using the National Handbook for Child Health Services and illustrations developed for CCHD. However, they were not trained in the CCHD approach. Received a traditional 120-minute lecture on child overweight and the BMI growth chart with no possibility for discussion or reflection.	Pg. 3 (1)

	Intervention: Received an interactive 1-day training in small groups focused on how to promote child participation and how to apply a family-therapeutic solution focused approach. Main components of training available (3). Was followed by 4 tutorial sessions of 1 hour every 2 months to reflect on how to build trust with the family, how to contextualize and re-frame into positive reflections and how to handle the challenges encountered in the early communication about weight.	Pg 3 (1), 823 (3)
How		
Describe the modes of delivery (e.g. face-to-face or by some other mechanism, such as internet or telephone) of the intervention and whether it was provided individually or in a group.	**Control & Intervention:** Started at the 4-year health visit, assumed to be in-person.	
Where		
Describe the type(s) of location(s) where the intervention occurred, including any necessary infrastructure or relevant features.	**Control & Intervention:** Occurred at 37 Child Health Centres in a southern county in Sweden.	Pg. 2 (1)
When and How Much		
Describe the number of times the intervention was delivered and over what period of time including the number of sessions, their schedule, and their duration, intensity or dose.	**Control:** A one-time visit at the 4 year old mark **Intervention:** 2 visits. A 10-minuted structured dialogue for the universal part, and a 45-minute Family Guidance visit. *?any other visits*	Pg 3-4 (1)
Tailoring		
If the intervention was planned to be personalised, titrated or adapted, then describe what, why, when, and how.	**Intervention:** The Family Guidance targeted session engaged family members in identifying protective factors or finding sustainable lifestyle changes.	Pg. 4 (1)
Modifications		
If the intervention was modified during the course of the study, describe the changes (what, why, when, and how).	None reported	
How Well		

Planned: If intervention adherence or fidelity was assessed, describe how and by whom, and if any strategies were used to maintain or improve fidelity, describe them.	Fidelity to the study protocol was monitored through the collection of data from the child's records on delivery of care for children with overweight at baseline and through a digital questionnaire containing 11 questionnaires distributed to all nurses at baseline.	Pg. 9 (1)
Actual: If intervention adherence or fidelity was assessed, describe the extent to which the intervention was delivered as planned.	Throughout the RCT, 26 nurses stopped working in the Child Health Services: 11 nurses in the control group and 16 nurses in the intervention group. The intervention group recruited 4 new nurses trained in CCHD, while 5 nurses could not be trained, resulting in the withdrawal of 285 children in the intervention group that met a nurse not trained in CCHD, of which 31 children had overweight.	Pg. 9 (1)

** **Authors** - use N/A if an item is not applicable for the intervention being described. **Reviewers** – use '?' if information about the element is not reported/not sufficiently reported.

1. Derwig, M., Tiberg, I., Björk, J., Welander Tärneberg, A., & Kristensson Hallström, I. (2021). A child-entered health dialogue for the prevention of obesity in child health services in Sweden - A randomized controlled trial including an economic evaluation . Obesity Science and Practice, 1–14.
2. Derwig M, Tiberg I, Björk J, Hallström I. Child-Centred health dialogue for primary prevention of obesity in child health services – a feasibility study. Scand J Public Health. 2021;49(4):384-392. https:// doi.org/10.1177/1403494819891025
3. Castor C, Derwig M, Borg SJ, Ollhage ME, Tiberg I. A challenging balancing act to engage children and their families in a healthy lifestyle – nurses' experiences of child-centred health dialogue in child health services in Sweden. J Clin Nurs. 2021;30(5/6):819-829. https://doi.org/10.1111/jocn.15622

Article Title	Can a dairy-rich diet be effective in long-term weight control of young children	
Authors	Kelishadi R, Zemel MB, Hashemipour M, Hosseini M, Mohammadifard N, Poursafa P.	
		Page Number

		(Location in Text)
Brief Name		
Provide the name or a phrase that describes the intervention.	NR	
Why		
Describe any rationale, theory or goal of the elements essential to the intervention	A longitudinal study on the effect of dairy intake in early childhood on the acquisition of body fat throughout childhood showed that suboptimal dairy intake during preschool was associated with greater gains in body fat throughout childhood. The objective of the current study was to determine the short- and long-term results of a randomized controlled trial of a dairy-rich diet on generalized and abdominal obesity, and the components of metabolic syndrome among obese pre-pubescent children.	Pg 602 (1)
What		
Materials: Describe any physical or informational materials used in the intervention, including those provided to participants or used in intervention delivery or in training of intervention providers. Provide information on where the materials can be accessed (e.g. online appendix, URL).	All participants attended family-centered education sessions on healthy lifestyle (no materials provided)	Pg. 603 (1)
Procedures: Describe each of the procedures, activities, and/or processes used in the intervention, including any enabling or support activities.	**All Groups:** All participants attended 6 consecutive monthly family-centered education sessions about healthy lifestyle (healthy nutrition and increasing physical activity) conducted by a pediatrician and nutritionist. **Dairy Rich Group (DR):** A dairy rich diet of >800 mg of calcium per day with no change in energy or macronutrient intake was recommended to the children of this group. Were recommended to obtain most of their calcium from low-fat and regular milk, cheese and yogurt, as well as liquid and sold curt. **Energy-Restricted Group (ER)**: Participated in similar nutrition education session as	Pg. 603 (1)

	the DR group. Were placed on a caloric restriction regimen with an energy content restricted to the caloric requirement for height. To ensure equal treatment intensity between the 2 intervention groups, the same team provided diet recommendations through educational sessions of the same number, duration and interval. **Control:** Received no dietary recommendations aside from what was discussed during the healthy lifestyle education sessions.	
Who Provided		
For each category of intervention provider (e.g. psychologist, nursing assistant), describe their expertise, background and any specific training given.	The healthy lifestyle education sessions were provided by a pediatrician and nutritionist It is not clear who provided the DR and ER groups education, or whether this was done in conjunction with the healthy lifestyle education sessions.	Pg. 603 (1)
How		
Describe the modes of delivery (e.g. face-to-face or by some other mechanism, such as internet or telephone) of the intervention and whether it was provided individually or in a group.	It is not stated whether the healthy lifestyle education sessions were provided as a group or individually. It is assumed that the groups were done in person, but not explicitly stated	
Where		
Describe the type(s) of location(s) where the intervention occurred, including any necessary infrastructure or relevant features.	The participants had been referred to the Obesity Research Clinic, Preventive Pediatric Cardiology Department, Isfahan Cardiovascular Research Centre (ICRC), a collaborating centre of the WHO in Isfahan, Iran.	Pg. 602 (1)
When and How Much		
Describe the number of times the intervention was delivered and over what period of time including the number of sessions, their schedule, and their	All participants attended 6 consecutive monthly education sessions. Not stated duration of the sessions, and whether the intervention groups education sessions	Pg. 603 (1)

duration, intensity or dose.	ran in conjunction with the general education sessions.	
Tailoring		
If the intervention was planned to be personalised, titrated or adapted, then describe what, why, when, and how.	None reported	
Modifications		
If the intervention was modified during the course of the study, describe the changes (what, why, when, and how).	None reported	
How Well		
Planned: If intervention adherence or fidelity was assessed, describe how and by whom, and if any strategies were used to maintain or improve fidelity, describe them.	Intervention adherence was monitored by retention and attrition rates. No strategies regarding fidelity were discussed.	Pg. 604 (1)
Actual: If intervention adherence or fidelity was assessed, describe the extent to which the intervention was delivered as planned.	120 children with obesity were randomized, and of these 95 participants completed the 36-month follow-up period (75%).	Pg 604 (1)

** **Authors** - use N/A if an item is not applicable for the intervention being described. **Reviewers** – use '?' if information about the element is not reported/not sufficiently reported.

References:

1. Kelishadi R, Zemel MB, Hashemipour M, Hosseini M, Mohammadifard N, Poursafa P. Can a dairy-rich diet be effective in long-term weight control of young children?. J Am Coll Nutr 2009;28(5):601–10.

Article Title	Efficacy of family-based weight control program for preschool children in primary care	
Authors	Quattrin T, Roemmich JN, Paluch R, Yu J, Epstein LH, Ecker MA.	
		Page Number

		(Location in Text)
Brief Name		
Provide the name or a phrase that describes the intervention.	Buffalo Healthy Tots	Pg 684 (1)
Why		
Describe any rationale, theory or goal of the elements essential to the intervention	Goal was to test the efficacy of an innovative family-based intervention program designed for treating overweight/obese children (aged 2-5 years) and an overweight parent in the primary care setting. Hypothesized that children and parents randomized to a family-based behavioural intervention would have greater reductions in BMI percentile and BMI as the contributions of parental eating habits and obesity status is well linked to their child's weight status.	661 (1)
What		
Materials: Describe any physical or informational materials used in the intervention, including those provided to participants or used in intervention delivery or in training of intervention providers. Provide information on where the materials can be accessed (e.g. online appendix, URL).	**Control & Intervention** – standardized letter updating parents on the progress of their child, not provided **Intervention:** Icon diary used for documenting physical activity and sedentary behaviours. This was not provided	661 (1)
Procedures: Describe each of the procedures, activities, and/or processes used in the intervention, including any enabling or support activities.	**Information Control:** ten 60-minute sessions over 6 months in which a consistent group leader delivery dietary, physical and sedentary activities education. Children were engaged in active games. Parents received 8 phone calls between meetings by an assigned coach. Were given a child weight loss goal of 0.5 to 1lbs/week. Follow-up regarding progress was done with a standardized letter (*not provided)* at 3 months and at a well-child visit at 6 months. **Intervention:** Same as the information control. The group leader also emphasized parenting and behavioural strategies to promote child and parent-behaviour change. Parents also attended a 1:1 meeting at the time of group sessions with an assigned coach who assisted parents in shaping behaviour goals. Parents were instructed to record child and parent food intake, as well as physical and sedentary	661 (1)

	behaviours using an icon-based diary (*not provided)*, this allowed for shaping of goals. Were asked to weight themselves and their child twice per week with a weight loss goal of 1-2 lbs/week.	
Who Provided		
For each category of intervention provider (e.g. psychologist, nursing assistant), describe their expertise, background and any specific training given.	**Information Control/Intervention:** trained group leader (*unclear role & training received*) as well as an assigned "coach" who provided mid-way point phone check ins (*unclear training of "coach"*) Pediatricians blind to the group assignments reviewed the child's progress at 3 and 6 month interval. Trained research assistant measured child's weight at every session	661 (1) 662 (1)
How		
Describe the modes of delivery (e.g. face-to-face or by some other mechanism, such as internet or telephone) of the intervention and whether it was provided individually or in a group.	**Control & Intervention** – Group sessions, presumed to be in-person with individual phone calls by coach in between meetings. **Intervention -** Same as control with the addition of a 1:1 meeting at each group session, presumed to be in person	661-662 (1)
Where		
Describe the type(s) of location(s) where the intervention occurred, including any necessary infrastructure or relevant features.	Primary care setting → unclear location	660 (1)
When and How Much		
Describe the number of times the intervention was delivered and over what period of time including the number of sessions, their schedule, and their duration, intensity or dose.	**Control –** 53 randomized to control. Had ten 60-minute sessions over 6 months (4 weekly, 2 bimonthly and 4 monthly). Received 8 phone calls between meetings by their assigned "coach". **Intervention -** 52 randomized to the intervention. Also had ten 60-minute sessons over 6 months (4 weekly, 2 bimonthly and 4 monthly). Received 8 phone calls between meetings by their assigned "coach". Also met 1:1 with their coach either before or after each of the meetings	661,663 (1)
Tailoring		

If the intervention was planned to be personalised, titrated or adapted, then describe what, why, when, and how.	**Intervention –** Parents attended 1:1 meetings with an assigned coach who assisted parents in shaping behavioural goals after having reviewed food/activity diaries.	662 (1)
Modifications		
If the intervention was modified during the course of the study, describe the changes (what, why, when, and how).	None reported	
How Well		
Planned: If intervention adherence or fidelity was assessed, describe how and by whom, and if any strategies were used to maintain or improve fidelity, describe them.	**Control & Intervention:** Group leaders were closely supervised by investigators. No other strategies were discussed regarding maintaining fidelity.	662 (1)
Actual: If intervention adherence or fidelity was assessed, describe the extent to which the intervention was delivered as planned.	None reported	

** **Authors** - use N/A if an item is not applicable for the intervention being described. **Reviewers** – use '?' if information about the element is not reported/not sufficiently reported.

References:

1. Quattrin T, Roemmich JN, Paluch R, Yu J, Epstein LH, Ecker MA. Efficacy of family-based weight control program for preschool children in primary care. Pediatrics 2012;130 (4):660–6.

Article Title	A pilot randomized controlled trial of a clinic and home-based behavioural intervention to decrease obesity in preschoolers	
Authors	Stark LJ, Spear S, Boles R, Kuhl E, Ratcliff M, Scharf C, et al. (2011)	
		Page Number (Location in Text)

Brief Name		
Provide the name or a phrase that describes the intervention.	LAUNCH – Learning about Activity and Understanding Nutrition for Child Health	134 (1)
Why		
Describe any rationale, theory or goal of the elements essential to the intervention	Developed a 6-month behavioural intervention for preschool obesity based on social cognitive theory and modeled on successful behavioural family-based intervention and modeled on successful behavioural family-based interventions for school-aged children. The program will specifically target preschool years, and was found to have a decrease in BMI z-score which was maintained at 12 months post-intervention. LAUNCH was designed to produce small decreases or stabilize the rate of children's weight gain, consistent with the current recommendations for treatment of preschool obesity.	134, 135 (1)
What		
Materials: Describe any physical or informational materials used in the intervention, including those provided to participants or used in intervention delivery or in training of intervention providers. Provide information on where the materials can be accessed (e.g. online appendix, URL).	*Standard of Care:* Pediatrician used a scripted manual (not provided) to delivery dietary and physical activity recommendations outlined by the AAP (article with recommendations referenced). Each family was given a 1-page healthy food and activity brochure created by the Collaboration for Health Ohio (http://www.healthyohioprogram.org/healthylife/nutri2/nutrikids2/ounce.aspx). *LAUNCH*: Pedometer and diet diary were used as feedback tools. Parents were given a 14-day supply of vegetables at each of the six clinic visits. A written manual (not provided) was used by the clinical psychologist to conduct the parent group.	135 (1) (2)
Procedures: Describe each of the procedures, activities, and/or processes used in the intervention, including any enabling or support activities.	*Standard of Care*: Dietary and physical activity recommendations (outlined by AAP) were provided by a pediatrician using a scripted manual. Each family met with the pediatrician for a one-time 45-minute visit to review the growth chart and BMI. Each family was given a 1-page handout on healthy food and activity. *LAUNCH:* 6-month intervention consisting of 2 phases. Phase 1 (intensive phase) was 12 weekly sessions alternating between group-based clinic sessions (parents and	

	child attending concurrent groups), and individual home visits. Phase 2 (maintenance) was 12 weeks of every other week sessions, alternating between group sessions in clinic and home sessions. Parent-group sessions addressed dietary education, physical activity and parenting skills. Separate sessions were done for snacks/beverages, breakfast/lunch and dinner. Sessions were also done on decreasing screen time, increasing physical activity. Education regarding child behaviour management skills was also taught. Parents were given a 14-day supply of vegetables at each of the six clinic visits. Children attended a group session concurrently – nutrition education done through various activities as well as completed 15 minutes of moderate to vigorous activity. In-home sessions supported generalization of clinic-taught skills to the home environment. Phase 2 (maintenance) focused on helping families continue to make/maintain changes in eating and physical activity.	
Who Provided		
For each category of intervention provider (e.g. psychologist, nursing assistant), describe their expertise, background and any specific training given.	*Control:* Pediatricians *LAUNCH:* The parent group during the intensive phase was conducted by a licensed clinical psychologist following a written manual. Child groups were conducted by a pediatric psychology postdoctoral fellow and a research coordinator. In-home sessions were conducted by psychology postdoctoral fellows. No mentioned of specific training for implementation of the intervention. Anthropometric measurements were done by trained personnel from the research centre → did not mention what training required.	135-136 (1)
How		
Describe the modes of delivery (e.g. face-to-face or by some other mechanism, such as internet or telephone) of the intervention and whether it was provided individually or in a group.	**Control**: Delivered via a clinic appointment in person **LAUNCH:** Group sessions for both parents and children were done in person. Home visits were individual and done in person. Unclear if the maintenance phase (phase	

	2) were done in-person or whether done as a group/individual session.	
Where		
Describe the type(s) of location(s) where the intervention occurred, including any necessary infrastructure or relevant features.	**Control:** Pediatrician office **LAUNCH:** Group sessions were done in a clinic location. Home sessions done in participant's houses. Did not specify where phase 2 occurred.	
When and How Much		
Describe the number of times the intervention was delivered and over what period of time including the number of sessions, their schedule, and their duration, intensity or dose.	**Control:** One 45 minutes session **LAUNCH:** Phase 1 consisted for 12 weekly sessions alternating between group sessions and home visits. Group sessions were 90 minutes in duration and covered 1 topic relating to diet and physical activity. In-home sessions lasted 60-90 minutes.	
Tailoring		
If the intervention was planned to be personalised, titrated or adapted, then describe what, why, when, and how.	**LAUNCH:** Unclear whether home visits were personalized to address clinic-taught skills	
Modifications		
If the intervention was modified during the course of the study, describe the changes (what, why, when, and how).	None reported	
How Well		
Planned: If intervention adherence or fidelity was assessed, describe how and by whom, and if any strategies were used to maintain or improve fidelity, describe them.	**Control –** The pediatricians followed a scripted manual **LAUNCH** – Parent group was conducted by a licensed clinical psychologist following a written manual. Inter-rater reliability was conducted by an independent coder for home food environment assessments. No other outcomes were reported to have this. No other information regarding planned fidelity	
Actual: If intervention adherence or fidelity was		

assessed, describe the extent to which the intervention was delivered as planned.	Not reported	

** **Authors** - use N/A if an item is not applicable for the intervention being described. **Reviewers** – use '?' if information about the element is not reported/not sufficiently reported.

References:

1. Stark LJ, Spear S, Boles R, Kuhl E, Ratcliff M, Scharf C, et al. A pilot randomized controlled trial of a clinic and home-based behavioral intervention to decrease obesity in preschoolers. Obesity (Silver Spring) 2011;19(1):134–41.
2. Spear BA, Barlow SE, Ervin C et al. Recommendations for treatment of child and adolescent overweight and obesity. Pediatrics 2007;120 Suppl 4:S254–S288.

Article Title	A Pilot Randomized Controlled Trial of a Behavioral Family-Based Intervention With and Without Home Visits to Decrease Obesity in Preschoolers	
Authors	Stark LJ, Clifford LM, Towner EK, Filigno SS, Zion C, Bolling C, et al. (2014)	

		Page Number (Location in Text)
Brief Name		
Provide the name or a phrase that describes the intervention.	Learning about Activity and Understanding Nutrition for Child Health (LAUNCH)	Pg 1002 (1)
Why		
Describe any rationale, theory or goal of the elements essential to the intervention	To address the unique aspects of treating obesity in preschoolers (food neophobia or unwillingness to try new foods, tantrumming peaks in this age group). Changing the diet of preschoolers would necessitate changing parents' interaction with their child and their home environment. Developed and piloted a behavioural intervention for preschool obesity modelled on successful behaviour family-based intervention for school-age children, while addressing the unique challenges of preschoolers. Sought to examine a version of LAUNCH that conforms to Expert Committee guidelines for Stage 3 intervention on the number of sessions, structure, and support compared with a condition similar to Stage 1 intervention.	Pg. 1002 (1)
What		
Materials: Describe any physical or informational materials used in the intervention, including those provided to participants or used in intervention delivery or in training of intervention providers. Provide information on where the materials can be accessed (e.g. online appendix, URL).	**LAUNCH-Home Visits (HV) & Clinic:** Required a diet diary, vegetables. LAUNCH-clinic required a home clean-out box. **Control:** A 1-page healthy food and activity brochure created by the Collaboration for Healthy Ohio (https://www.nationwidechildrens.org/specialties/center-for-healthy-weight-and-nutrition/services-we-offer/ounce-of-prevention)	Pg. 1003, 1005 (1)
Procedures: Describe each of the procedures, activities, and/or processes used in the intervention, including any enabling or support activities.	**LAUNCH-HV**: Manualized intervention consisting of two phases (Intensive & Maintenance), alternating between group clinic and individual home sessions. *Phase 1:* Months 1-3 targeted lifestyle behaviour modifications and improving parenting skills (supplementary material available online for treatment flow and session topics). Weeks 2-7 included education on diet, weeks 8-12 education on physical activity and parenting skills at all visits. Parents kept a 7-day diet diary for themselves and their child. They were also provided with vegetables at each session	Pg. 1003-1005 (1)

	for daily taste tests between sessions. Children participated in a concurrent manualized group-based intervention paralleling the topics covered in the parent sessions focusing on education about healthy education, opportunities to try new foods and engage in physical activity. Home sessions followed a manualized protocol to support generalization of clinic-taught skills to the home environment through instruction, therapist modeling and parent rehearsal of dietary changes, physical activity and behaviour techniques. "Home clean-outs" were conducted following each clinic session on diet. *Phase 2:* Months 4-6 focused on helping families maintain treatment gains by engaging parents in long-term planning, problem-solving and use of parenting skills to promote maintenance of diet and activity changes. **LAUNCH-C:** Intervention content was identical to LAUNCH-HV clinic sessions. Instead of home-visits, parents were provided a "home clean-out" box to use on their own to eliminate high-calorie low-nutrient foods from the home. **Standard of Care:** Manualized intervention designed to deliver dietary and physical activity recommendations outlined by the AAP (2). Pediatrician explained BMI, BMI percentile and reviewed the child's growth chart. Pediatrician recommended screen time ≤ 2 hours; active play ≥ 60 mins daily, eliminating soda and ≤ 4 oz daily; fruits and vegetables ≥ 5 servings daily, limiting eating out; and appropriate portion size for preschoolers. They were also given a 1-page healthy food and activity brochure (3).	
Who Provided		
For each category of intervention provider (e.g. psychologist, nursing assistant), describe their expertise, background and any specific training given.	**LAUNCH-HV:** Parent-group clinic sessions were conducted by a licensed clinical psychologist or 2nd year psychology postdoctoral fellow. The kid groups were led by a pediatric psychology postdoctoral fellow and a research coordinator. Home sessions were conducted by a pediatric psychology fellow.	Pg. 1002-1003 (1)

	LAUNCH-C: Same as the LAUNCH-HV **Standard of Care:** Pediatrician	
How		
Describe the modes of delivery (e.g. face-to-face or by some other mechanism, such as internet or telephone) of the intervention and whether it was provided individually or in a group.	**LAUNCH-HV**: The education sessions were provided in the clinic as a group. The home-visits were individual. **LAUNCH-C:** Education sessions were in-person group sessions. **Standard of Care:** Individual in-person sessions.	Pg. 1002-1003 (1)
Where		
Describe the type(s) of location(s) where the intervention occurred, including any necessary infrastructure or relevant features.	Conducted at the Cincinnati Children's Hospital Medical Centre as well as home visits (LAUNCH-HV)	Pg. 1003 (1)
When and How Much		
Describe the number of times the intervention was delivered and over what period of time including the number of sessions, their schedule, and their duration, intensity or dose.	**LAUNCH-HV:** 18-sessions, 6-month intervention consisting of two phases. Phase 1 was 3 months in duration consisting of 12 weekly sessions alternating between group-based clinic sessions and individual home visits. The parent group sessions were 90 minutes in duration. Home visits lasted 6-90 minutes. Phase 2 consisted of 12 weeks of every-other-week sessions alternating between group clinic sessions and individual home visits. **LAUNCH-C:** Sessions were conducted every other week during months 1-3 and monthly during months 4-6 of the intervention, for 10 treatment sessions. **Standard of Care:** Pediatrician met with each family individually for one 45 minutes visit.	Pg. 1003-1005 (1)
Tailoring		
If the intervention was planned to be personalised, titrated or adapted, then describe what, why,	**LAUNCH-HV/C:** The maintenance phase focusing on helping to maintain treatment gains focused on problem-solving around individual barriers, and use of parenting	Pg. 1004 (1)

when, and how.	skills.	
Modifications		
If the intervention was modified during the course of the study, describe the changes (what, why, when, and how).	None reported	
How Well		
Planned: If intervention adherence or fidelity was assessed, describe how and by whom, and if any strategies were used to maintain or improve fidelity, describe them.	No planned fidelity assessments were discussed. Intervention adherence was monitored via attrition	Pg. 1006 (1)
Actual: If intervention adherence or fidelity was assessed, describe the extent to which the intervention was delivered as planned.	15 families were randomized to LAUNCH-HV, 14 to LAUNCH-C, and 13 to standard of care. Of those, 3 families in LAUNCH-HV, 1 in LAUNCH-C and 1 in standard of care, dropped out before completing all the treatment sessions. The 1 in standard of care dropped out after completing the session.	Pg. 1006 (1)

** **Authors** - use N/A if an item is not applicable for the intervention being described. **Reviewers** – use '?' if information about the element is not reported/not sufficiently reported.

References:

1. Stark LJ, Clifford LM, Towner EK, Filigno SS, Zion C, Bolling C, et al. A pilot randomized controlled trial of a behavioral family-based intervention with and without home visits to decrease obesity in preschoolers. J Pediatr Psychol 2014;39(9):1001–12.

2. Spear, B. A., Barlow, S. E., Ervin, C., Ludwig, D. S.,Saelens, B. E., Schetzina, K. E., & Taveras, E. M.(2007). Recommendations for treatment of child and adolescent overweight and obesity. Pediatrics, 120(Suppl. 4), S254–S288. doi:10.1542/peds.2007-2329F

3. Toolkit (2007). Ounce of prevention is worth a pound: Preventing childhood obesity starting from birth. Retrieved December 26, 2021. https://www.nationwidechildrens.org/specialties/center-for-healthy-weight-and-nutrition/services-we-offer/ounce-of-prevention